

Stress Urinary Incontinence — Male

WARWIKI

Patient Information · Leaking after prostate surgery — with effort or activity

Stress urinary incontinence (SUI) in men means leaking urine when something puts sudden pressure on the bladder — coughing, standing, lifting, or activity. In men it is almost always due to weakening of the urinary control muscle (sphincter) after **prostate surgery** or radiation. It often improves over the first year, and when it doesn't, very effective treatments are available.

About This Condition

After the prostate is removed, the main **urinary sphincter** (the valve that holds urine in) does most of the work, and it may be weakened. The result is leaking with effort — from a few drops needing a pad, to heavier leakage. This is different from **urge** leakage (a sudden need to go), which is treated differently; some men have both.

What Causes It

- **Radical prostatectomy** (prostate removal for cancer) — the most common cause
- Other prostate procedures or **pelvic radiation**
- Less often, nerve or bladder conditions

Leakage is usually **worst early** and improves over weeks to months as healing and pelvic-floor strength return.

LEARN THE TERMS

Stress incontinence

Leaking urine with pressure — coughing, standing, lifting.

Sphincter

The muscle valve that holds urine in until you choose to go.

Prostatectomy

Surgery to remove the prostate, the usual cause of male SUI.

Pelvic floor

The muscles you train to help control urine.

Pad test

Counting pads per day to measure how much you leak.

Male sling

A support placed under the urethra for mild–moderate leakage.

Artificial urinary sphincter

An implanted device (a cuff you control) for moderate–severe leakage.

WILL IT GET BETTER ON ITS OWN? Often, yes — leakage after prostate surgery usually improves over the first 6–12 months, especially with pelvic-floor exercises. If bothersome leakage persists past about a year, that is the point to consider a sling or an artificial sphincter, both of which work very well.

How It's Diagnosed

- A talk about when and how much you leak (often counted as **pads per day**)
- A **urine test**, and a check of how well you empty
- Before surgery: a look in the urethra (cystoscopy) and sometimes bladder-function testing

How It's Treated (Step by Step)

- 1 **Pelvic-floor exercises** (Kegels), ideally started around the time of surgery, plus managing fluids and weight.
- 2 **Watchful improvement** over the first year, with pads/protection as needed.
- 3 **Male sling** — for persistent mild-to-moderate leakage (nothing to operate after).
- 4 **Artificial urinary sphincter (AUS)** — the gold standard for moderate-to-severe leakage, and the choice after radiation. Each has its own handout.

Living With Male SUI

- Do pelvic-floor exercises consistently — they are the foundation.
- Use pads or a protective garment for confidence while you recover.
- Limit evening fluids and caffeine if night-time leakage is an issue.

Call your team if you have:

- Burning, fever, or **blood in the urine** (possible infection)
- You suddenly **cannot urinate**
- Leakage that is worsening rather than improving

THREE THINGS TO REMEMBER

1. Male SUI is almost always from a weakened sphincter after prostate surgery or radiation — and it usually improves over the first year.
2. Pelvic-floor exercises are the foundation; for lasting leakage, a male sling or artificial urinary sphincter works very well.
3. Give recovery up to a year before deciding on surgery. Call for fever, blood in the urine, or being unable to urinate.